VITA

Life in a Zone of Social Abandonment

UPDATED WITH A NEW AFTERWORD AND PHOTO ESSAY

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Photographs by Torben Eskerod

"Reads, in the best of ethnographic fashion, like a mystery thriller."
Journal of the Royal Anthropological Institute

Part Two

CATARINA AND THE ALPHABET

Life of the Mind

As we passed through the gate of the infirmary, my eyes immediately turned to a woman seated in a wheelchair in the shade. She was writing. "It's Catarina," I told my wife, Adriana. This time, Catarina was no longer riding her bicycle. Death was coming upon her, I thought.

With her head down, Catarina held a pen and scribbled with much effort. We greeted her by name, and she looked up, recognizing us. "João and Adriana," she said.

Catarina seemed dazed; she spoke slowly and with great difficulty, as if she had suffered a stroke. We asked how she was doing. "My legs don't help anymore," she replied, adding that this problem was a result of "rheumatism," although she was not taking any medication for it. "Sometimes the volunteers give me pills, but I don't know what they are."

What are you writing?

"This is my dictionary," she said. "I write so that I don't forget the words. I write all the illnesses I have now, and the illnesses I had as a child."

Catarina handed me her book. Her handwriting was uneven. The words were composed in block letters, with no cursive writing, and with few verbs or full sentences. I was amazed by the force of the words, by her ragged poetry:

Divorce
Dictionary
Discipline
Diagnostics

Marriage for free
Paid marriage

Operation
Reality
To give an injection
To get a spasm
In the body
A cerebral spasm

Why do you call it a dictionary?

"Because it does not require anything from me, nothing. If it were mathematics, I would have to find a solution, an answer. Here, there is only one subject matter, from beginning till the end. . . . I write it and read it."

I perused the dictionary, as Adriana spoke with Catarina. "In the womb of pain," she had written. "I offer you my life." "The present meaning." Amid recurring references to medical consultations, hospitals, and public notaries, she wrote of a working woman and wanderer, of sexual emotion and mental disturbance, of medication and food for a baby, of poverty and abundance, of officers and indebtedness, and of things being "out of justice."

Blended with allusions to spasms, menstruation, paralysis, rheumatism, paranoia, and the listing of all possible diseases from measles to ulcers to AIDS were names such as Ademir, Nilson, Armando, Anderson, Alessandra, Ana. Here and there, she wrote of motherhood; of divorce; of a rustic life with sows and insects, veterinarians and a Rural Workers Association; and of desire. Striking statements from a world that was no more.

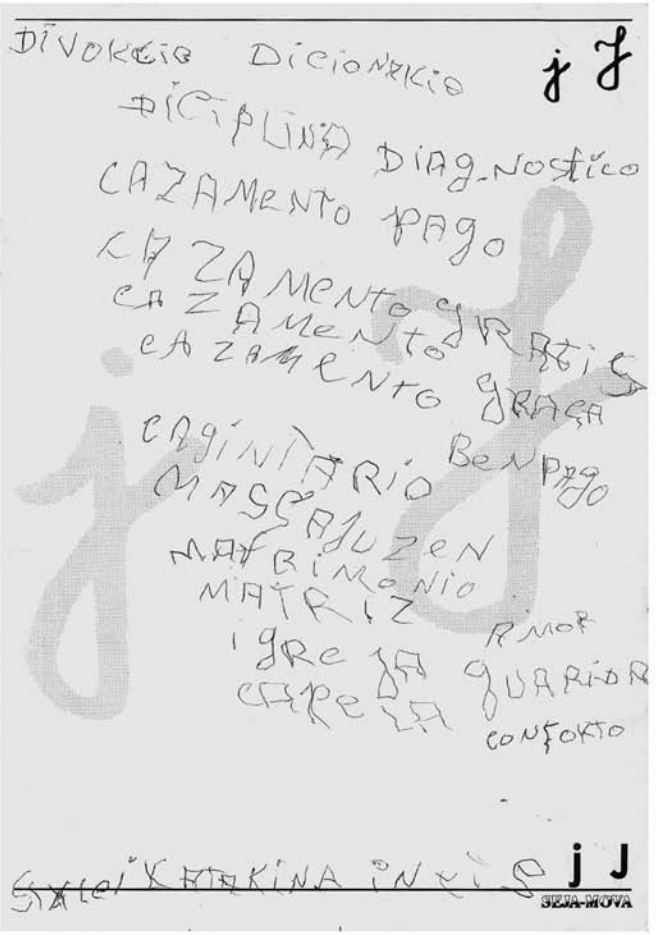
Question, answer, problem to solve, the head
Who contradicts is convicted
The division of bodies

Then there were expressions of longing:

Recovery of my lost movements
A cure that finds the soul
The needy moon guards me

With L I write Love
With R I write Remembrance

Catarina writes to remain alive, I told myself.[33] These are the words that form her from within. What are the ways of these words? How to tell it all, and what are we to do with it?


DIVOKCIO DICIONAKIO
j J
DICIPLINA DIAG-NOSTICO
CAZAMENTO PAGO
CAZAMENTO GRATIS
CAZAMENTO GRAÇA
CAGINTARIO BEN PAGO
MASCAJUZEN
MATRIMONIO
MATRIZ
AMOR
IGREJA GUARIDA
CAPELA CONFORTO
SYLOIKATAKINA INEIS
j J
SEJA-NOVA

Society of Bodies

I returned to Vita a week later, this time alone. With neighborly care, Catarina immediately asked, "Where is Adriana?" She remarked that she had enjoyed talking to both of us the other day. I sensed in her words an integrity that neither forgot bonds nor envied the bonds of others, the character of a time when one earned respect, if not from governmental institutions and employers, then at least from family members and neighbors. As simplistic as it may sound, this sociality was a life-giving force.

The left side of Catarina's face was bruised. "I fell from the chair when I tried to reach the bathroom." A volunteer overheard and contradicted her, claiming that she had thrown herself to the ground in a fit of rage.

Why aren't you writing today?

"I already filled the dictionary."

I asked if I could take another look at it. Catarina called India, a silent, twenty-something woman of Indian descent who was sitting on a bench nearby, and asked her to fetch the notebook, which was wrapped in a plastic bag inside a suitcase under her bed. She assured India: "It's not locked." As the story goes, India is "mentally retarded." Her brother, they say, is a Pentecostal pastor who speaks about her on a radio program but never comes to see her.

I asked Catarina if she wanted to keep writing. When she responded, "Yes," I ripped out several pages of scribbled field notes from my own notebook and gave the empty book to her. "Write your name and address here," she asked.

"I want to leave, I want to leave," a young black man named Marcelo broke into the conversation. Like the majority of those in Vita, his true name and

origins were either unknown or did not matter. He kept looking straight into my eyes, his hands grasping a small suitcase: "Take me, take me with you."

Many *abandonados*, like crippled Iraci, have no formal identification but recall a home, a family, a childhood, or simply freedom in the streets. "I also want to leave," he told me. "I came from Lages, state of Santa Catarina. I was raised in the interior and like it better than the city. I lost my father and my mother. We had cows and pigs and planted corn and beans. I have ten siblings, all scattered. My sister put me on a bus to Porto Alegre. Nobody wanted to take care of me. I was already paralyzed. I got paralyzed when I was one-and-a-half years old. I lived in the streets for five years. Now I am forty-one years old and have been here for more than five years. Better to live in the streets than in a place like this for the rest of life."

Vita "makes me nervous." Iraci said. "Here one dies." He has seen too many pass away. "During winter, it is pretty bad. I lost count of how many died this past year. It's serious. This place is a sadness. I want to get out of here. This is not life. It's the end of life. The one who is ill gets even more ill, and one gets nervous. I am a nervous man."

What happens to the dead?

"When someone dies, the administration calls the morgue. They pick up the body and put it in the machine."

What do you mean?

"They throw oil over the body and set the body on fire. Then it becomes ashes, and the ashes are thrown into the Guaíba River. If one is buried, it is only for a few days, for they need the grave for other people. That's what I heard."

During the long days in Vita in which nothing happens, Iraci and his friends, including India, whom he says he is dating, "keep track of time. . . . We tell each other which day of the month and which year it is, the year that passed and the year that will begin. One reminds the other. Today is December 30, 1999; tomorrow is December 31, 1999, right? See how smart I am? Thank God, my head works very well. I am not ill."

I asked Iraci what was inside the plastic bag over his lap.

"The words of God."

So you know how to read?

"No, but I understand them."

What do the words of God say?

"They say, 'The Lord is my shepherd, and I will lack nothing.' "

What else do you carry in here?

"Bread. I keep the crumbs I find so that I can eat during the day. Sir, I enjoyed talking to you very much. Do you have a pen?"

Yes.

"So, I want you to write my name in your book. It's Iraci Pereira de Moraes. My name comes from my deceased mother. Her name was Dormíria Pereira de Moraes. The name of my deceased father was Laudino Pereira de Oliveira."

Why didn't you get your father's name?

"I don't know why. I was pushed to the side of the deceased mother. I think it's good like this."

Were you registered at birth?

"Yes, but I lost my documents in the streets. I must do them all over again."

Iraci then again summed up his fate: "I tell you the truth, I have no father or mother. I have ten siblings. We were eleven; one died. We are scattered in the world. So I wanted to see if I could return to the interior. I have an acquaintance, a good friend of the family. He lives near the town of Arvelino Carvão."

Catarina was listening. We resumed our conversation. Why, I asked her, do you think families, neighbors, and hospitals send people to Vita?

"They say that it is better to place us here so that we don't have to be left alone at home, in solitude . . . that there are more people like us here. And all of us together, we form a society, *a society of bodies.*" And she added: "Maybe my family still remembers me, but they don't miss me."

Catarina had condensed the social reasoning of which she was the human leftover. I wondered about her chronology and about how she had been cut off from family life and placed in Vita. How had she become the object of a logic and sociality in which people were no longer worthy of affection, though they were remembered? And how was I to make sense of these intimate dynamics if not by trusting her and working through her language and experience?

India could not find the dictionary, so she brought the whole suitcase. A strong smell of urine, moisture, and also a kind of sweetness wafted out as I opened it. The contents of the suitcase were all that Catarina had in life: a few pieces of old clothing, some carefully assembled candy wrappers, fake jewelry, a bottle of cheap powder, a toothbrush, and a comb as well as several plastic bags containing magazines, books, and notebooks. "My worker's identity card was kept by the hospital," she noted.

I picked up the dictionary and read aloud some of her free-associative inscriptions.

Documents, reality
Tiresomeness, truth, saliva
Voracious, consumer, saving, economics
Catarina, spirit, pills
Marriage, cancer, Catholic church
Separation of bodies, division of the estate
The couple's children

The words indexed the ground of Catarina's existence; her body had been separated from those exchanges and made part of a new society.

What do you mean by the "separation of bodies"?

"My ex-husband kept the children."

When did you separate?

"Many years ago."

What happened?

"He had another woman."

She shifted back to her pain: "I have these spasms, and my legs feel so heavy."

When did you begin feeling this?

"After I had Alessandra, my second child, I already had difficulty walking. . . . My ex-husband sent me to the psychiatric hospital. They gave me so many injections. I don't want to go back to his house. He rules the city of Novo Hamburgo."

Did the doctors ever tell you what you had?

"No, they said nothing."

Denial or resistance to the verdict "psychotic," I thought. But what was the heaviness in her body that she repeatedly described? Just as they had the first time we met, in March 1997, Catarina's words suggested that something physiological had preceded or was related to her exclusion as mentally ill, and that her condition worsened in medical exchanges. "I am allergic to doctors. Doctors want to be knowledgeable, but they don't know what suffering is. They only medicate."

Oscar, the infirmary's coordinator, stopped by. Every time the good man saw me, he called me "*O vivente*"—"O living creature"—an expression I found eerie in that context. As we moved aside, Oscar explained that, as far

as he knew, a hospital had sent Catarina to Vita because her family did not want to care for her. But he had no specific information to back this up.

"She is very depressed. Like the others, she feels rejected and imprisoned here. They are placed here, and nobody visits them." He linked Catarina's paralysis to a complicated labor, "a woman's problem. She lost the child, it seems. We don't know which hospital it happened in; we have no reports. These are things people say, but nobody truly knows. The fact is that people make pacts to get rid of the person, and that's why we have institutions like Vita. That's how things work these days."

Oscar emphasized that probably 80 of the 110 people now in the infirmary (the population had significantly decreased since the time of Zé das Drogas's administration) were "psychiatric cases." But he agreed that Catarina was "a lucid person." A person that no one listens to any longer, I added.

"When my thoughts agreed with my ex-husband and his family, everything was fine," Catarina recalled, as we resumed the conversation. "But when I disagreed with them, I was mad. It was like a side of me had to be forgotten. The side of wisdom. They wouldn't dialogue, and the science of the illness was forgotten. My legs weren't working well. The doctors prescribe and prescribe. They don't touch you there where it hurts. . . . My sister-in-law went to the health post to get the medication for me."

According to Catarina, her physiological deterioration and expulsion from reality had been mediated by a shift in the meaning of words, in the light of novel family dynamics, economic pressures, and her own pharmaceutical treatment. Her affections seemed intimately connected to new domestic arrangements. "My brothers brought me here. For some time, I lived with my brothers . . . but I didn't want to take medication when I was there. I asked: why is it only me who has to be medicated? My brothers want to see production, progress. They said that I would feel better in the midst of other people like me."

But Catarina resisted this closure, and, in ways that I could not fully grasp at first, she voiced an intricate ontology in which inner and outer state were laced together, along with the wish to untie it all: "Science is our consciousness, heavy at times, burdened by a knot that you cannot untie. If we don't study it, the illness in the body worsens. . . . Science . . . If you have a guilty conscience, you will not be able to discern things."

Catarina said that she wrote in order "to know afterward," as if she could not have been present in the circumstances that determined the course of her existence. Her spoken accounts and her writing contain the confused

sense of something strange happening in the body—"cerebral spasm, corporal spasm, emotional spasm, scared heart." Along with all those people coming in and out of her house, and her moving from house to hospital to other houses, it seemed there was a danger of becoming too many, strange to herself. "One needs to preserve oneself. I also know that pleasure in one's life is very important, the body of the Other. I think that people fear their bodies."

Writing helped her to endure the days in Vita, Catarina added. "Now and then, we also talk to each other. But the toughest are the nights, for then we are all alone, and one desire pushes the other. I have desire, I have desire."

When I approached Iraci to say good-bye, he repeated that he and India were dating. "We take care of each other. Last night, I dreamed it was our wedding, and we were eating the cake. Then I woke up . . . and I was so hungry." But the story is much more complicated. I learned that volunteers sometimes actually tied India to her bed to prevent her from masturbating in public. I also heard rumors that volunteers from the recovery area whose task it was to bathe the women had forced themselves sexually on India.

As I bade Catarina farewell, I asked to borrow the dictionary, for I wanted to study it. She consented. I told her that I would bring it back in August, and we would continue the conversation. She smiled and said that she had run out of ink. "I need another pen."

I read the word she was trying to finish writing: CONTACT.

Iraci and India, Vita 2001

Inequality

"A maimed statue." *Estátua entrevada.* That is how Catarina described her condition in her dictionary. *Entrevada* means to be paralyzed; it also means to become dark or obscure, to grow clouded. The associations that follow this description are striking—in the eyes of the maimed statue, there is Catarina, along with her son, confronting officers and looking into the eyes of a machine:

Birth certificate
Catarina and Anderson
To be present in person
Policeman
Electoral officer
Eye to eye
Machine
To make meaning

On the next page of the dictionary, Catarina repeats the word "statue" and, writing in the imperative, demands to be addressed: "Call my address: *Brasil*, *Brancil*, *Brecha*, *Brasa*."

Vita is Brazil's address or destination. *Brancil* is a word Catarina made up; to me, it sounds like the name of a prescription drug. *Brecha* is a fissure, a wound, as well as the void that Catarina became. *Brasa* is a burning coal. It also suggests anxiety, wrath, and sexual tension.

The notebook in which Catarina wrote her dictionary had been distributed a few years earlier by the municipal government of Porto Alegre, considered a model of popular administration both domestically and interna-

tionally (Pont and Barcelos 2000; Abers 2000). The site of the World Social Forum, the city has become famous for its policies of social inclusion, most notably its "participatory budget-making." The inside covers of the notebook outlined the consciousness-raising philosophy of the Workers' Party under the title "Writing (Dis)Organizes Life": "You, the people, are the main actors in the work we carry out. . . . Information invades our lives without asking for permission. All forms of the written word are a daily part of the city, but citizens are increasingly excluded from the content. This reality is in constant flux, blurring the conceptual lines between literacy and illiteracy. How much time is needed before we learn to critically engage with the written word?"

In this official text, the city's secretary of education observed that at least twenty million people in the country were illiterate (some 15 percent of the population) and added that access to literacy and education was "a political project that questions the neoliberalism being implemented in the country and in our province. The democratic and popular government of Porto Alegre, in partnership with civil society, ensures the high quality of education and also guarantees access to health care, employment, leisure, sanitation, and housing." The secretary concluded by explaining that this project, called "totalities of knowledge," took its central inspiration from Paulo Freire's argument that *reading the world precedes reading the word:* "Our literacy program develops critical citizens who have a choice, with the capacity to transform their own lives and the realities of the world."

Catarina's use of this discarded educational material to compose her dictionary ironically exposes the imaginary and selective quality of what is described here as social change. The truth is that Catarina and the residents of Vita remain excluded from this particular popular project as well as most others.

"Citizens are those who search for services," explained Mariane Gross, a journalist and human rights activist working in the city's security office, who was critical of what the policies of social inclusion have meant in practice. She argued that the Workers' Party administration had been highly effective in creating novel "service counters," which address various medical and social needs, within a limited capacity. This in turn generated a new culture of citizenship and "democratic experts," as she put it. "Those who want access have to get registered, wait in line, and participate. But what if you don't read the folder, if you don't have friends who tell you about these possibilities? Individuals have also learned to use this structure to accumulate power in their communities." Meanwhile, "on the way to the counter, others, par-

ticularly the young and unemployed, are recruited by parallel forms of commerce and government—that is, organized crime."

In 1997, I presented some of my initial ethnographic findings from Vita at an AIDS workshop organized by Porto Alegre's municipal government. I suggested that there were signs of a hidden and untreated AIDS epidemic in Vita, which could well be an indication of what was happening in Porto Alegre's streets and ghettos (Biehl 1999b). At the time, a representative of the Ministry of Health voiced indignation about "such a degree of dehumanization" and asked the local officials present to consider "closing Vita in the name of public health." The city's health secretary promised that her office would definitely investigate.

But, as another top city administrator admitted, the pressure to produce quick results for this progressive administration too often led to the creation of commissions and the writing of reports: "In truth, problems are identified, but things are not solved." The poorest urban inhabitants, by and large, remained in a "vacuum of response." And, in this vacuum, new social units and economic activities emerged to care for the invisible. This was tragically apparent in the so-called geriatric houses that mushroomed throughout the city to shelter the elderly, the mentally ill, and the disabled—the "unproductive and useless," as Mariane Gross described them. "We used to say that in each street of Porto Alegre there was a clandestine hospice, operating without legal authorization."

In 1998, Gross began a campaign to publicize the tragic conditions in these institutions. "People are confined and have no adequate care. Some of these businesses are surrounded by barbed wire, like camps." On July 2, 1999, for example, a fifty-eight-year-old man was bitten to death by dogs in a geriatric house in Porto Alegre. "Bits of skin were all over the ground," Gross and her colleagues wrote in the annual report of the state's Human Rights Commission (Comissão de Direitos Humanos 2000:108).

But human rights rhetoric was not strong enough to close down Vita and similar institutions. The city's public health inspection service had also begun to investigate these businesses but was having difficulty finding judges who would support shutting them down, according to health professional Jaci Oliveira. "The judges tell us that these houses are doing good. After all, where would these people go if they were freed?"

Even if Vita had been shut down, it would most certainly have reemerged elsewhere in the city. For Vita is indeed symbiotic with various levels of government, and people like Catarina now have their destinies forged by a set of forces and a logic of exceptions that operate, in her words, "*out of justice*."

Ex-Human

"I finished writing the book you gave me," Catarina reported when we met again in early August 2000. "I left the book in the pharmacy with Clóvis, the nurse, but he threw it away. I was sad. I kept thinking that one day João and Adriana will come back, and they will want to read the book, and I don't have it anymore." I told Catarina I trusted that her writing would resume. She then confided: "Clóvis and I are dating."

She quickly changed the subject: "My little suitcase was also thrown away. The volunteers said that it was getting moistened."

I handed Catarina another empty notebook. She smiled, with a seemingly sedated face: "I have been at a standstill. . . . My head was full, full of nonsense. . . . So I stopped writing."

"My gums are inflamed. It aches a lot. Clóvis told me that they would take me to the dentist." Catarina added that he was giving her vitamins and pills, a white and a blue one, for pain. "Clóvis gives medication to each one of us. He puts the pills in the little cups with our names and the dose, the right dose . . . and distributes it to all of us." Catarina looked very tired.

How did you sleep?

"I woke up in the middle of the night because of Lili, my bedmate. She talks in her sleep. It took me a long time to fall asleep again."

Do you recall any dreams you had?

"I dreamed that I was . . . no . . . suddenly a man came and hit me and pulled my hair. I don't know. I felt bad and began to scream, asking for help. Then Lili was there. I don't recall more. It was a nightmare."

It sounded like it had really happened, I thought. I asked whether she had recognized the man in the dream, but she said she had not.

I mentioned that sometimes, after waking from a dream, I would scribble down the things I recalled.

"Yes," she replied, "dreams help us to understand the fears we have. A nightmare can also be a desire. If one doesn't study what one has dreamed, then the dreams stay in the life that was dreamed. And, as one returns to life, one keeps thinking that everything is normal."

I was confused. Are you saying, I asked, that if we don't interpret a dream, then the dream stays present during the day, as if we lived in a dream-state?

"No, that's not what I am saying. A little remnant of the dream is transmitted to us . . . the rest is up to us to channel and decipher. If we don't decipher, we will not be able to remember what actually happened, what was and what wasn't."

What was and what wasn't. In Catarina's conception, the workings of the unconscious did not simply substitute for reality.[34] Rather, the unconscious seemed to be a storehouse of ciphers that one must assemble and decode in order to understand *what actually happened*—the truth of losing one's way of being in the world.

A cipher is an arithmetic symbol—zero—of no value by itself, used to occupy a vacant place in decimal numeration. A cipher is a person or thing that fills a place but is of no importance, a nonentity. A cipher is also a secret or disguised system of writing, a code used in writing; or a message written in this manner; or a key to such a system.

According to Catarina, that which truly happened continues to exist in the lost and valueless, in nonentities such as herself. Our grammars, George Steiner writes, make it difficult, even unnatural, to phrase a radical existential negativity, "but the failure of the human enterprise makes the doubt inescapable" (2001:39).

How did you learn to decipher your dreams?

"By myself," answered Catarina, "when I was little. As I woke up, I kept thinking about what I had seen in the dream. I kept it to myself. I learned that it is good to dream, to think, the mind . . . "

But the time came when the borderline between the I and the Other had to be drawn more clearly. The fragile and forceful birth of writing and of the social person was what she described next.

"My father, he taught me the alphabet. We sat at the kitchen table, and he wrote 'a,' 'b,' 'c' in my notebook. I had to memorize the letters. But at first they didn't stay in my mind. My father kept saying, 'Catarina, you must keep this in your mind. If not, you will know nothing, and you won't be a person.' It was difficult. I wept a lot, but I learned the a, the b, and the c."

That is what her dead father, she said, had left her with. A vital supplement, I thought.

Without knowing why, I changed the subject. Reminding Catarina that she had once told me that the doctors could not understand her pain, I asked her when she had first visited a doctor. She took my question literally, not letting me divert the conversation from where her thinking was taking it: "I think I was five years old. . . . I came down with something on my body that itched a lot. So my father took me to the pharmacist, and I healed. As a child, I only went to pharmacies. Real doctors who wrote prescriptions, that was later, the psychiatrists."

She was telling me more than I could comprehend at the time about a painfully learned symbolic order and a personhood, both of which were no longer of any value (like her discarded book). As she alluded to them, the pharmaceutical identifications were another kind of writing that seemed to stand for her estrangement from the world.

"My ex-husband first took me to the psychiatrist, Dr. Gilson, in Novo Hamburgo, for him to help me, to discover the illness. But he lied to the doctor, saying that I was aggressive, that I beat the children. I got so mad that I actually hit him in front of the doctor. The doctor only prescribed. The nurse gave me an injection. I always had some medication to take. They said that they wanted to heal me, but how could they if they did not know the illness? . . . If I will count on doctors to tell me what I feel, I will remain incapacitated, because they don't know the reason of what I am, of my illness, of my pain. They don't know a thing."

At that point, a woman who had been observing us for a while at a distance finally approached.

"This is my friend Lili," said Catarina. "She speaks at night."

"Yes, I am her friend," replied Lili, looking straight into my eyes, with a wide-open smile and a copy of the New Testament in her hands.

Before I could ask Lili anything, she asked me: "Do you know what 'not to live by the desire of the flesh any longer' means?"

Stunned by her question, I failed to answer. I stayed on the surface of her words and mentioned that, as far as I knew, this was a statement by the Apostle Paul and that I would look into it further. I asked Lili where she was from, a question I grew up learning to ask in that world where mobility, both geographic and social, was all too rare.

"I am from Canoas, but now I am living here. I used to be Catholic, but then I converted to the Assembly of God. I used to leave home and run to the church. My husband hit me. I lived in the streets for some time, but

then I went to live with my son. My son brought me here. My daughter-in-law wanted to kill me. I did nothing wrong. She called him 'daddy,' and I told her that I did not like that. She then tried to kill me with a kitchen knife."

I was again faced with a condensed account of what the "mad person" thought had happened to her in life. I had heard similar accounts from Catarina when I first met her in 1997 and from Iraci during my previous visit. In their initial statements, all three had described being banned from the family and suffering the rupture of relations as well as the dangerous and now impossible desire for homecoming. These were not illness narratives channeling a search for meaning (Kleinman 1988; Good 1994; Mattingly 1998). Nor were they the "schizophrenic recording codes" that Deleuze and Guattari saw as opposing or simply parodying social codes, "never giving the same explanation from one day to the next" (1983:15). Neither were they the "diffuse and external rain of distractions" that marked the being-in-the-world of the homeless in the Boston shelter chronicled by Robert Desjarlais (1994:897).

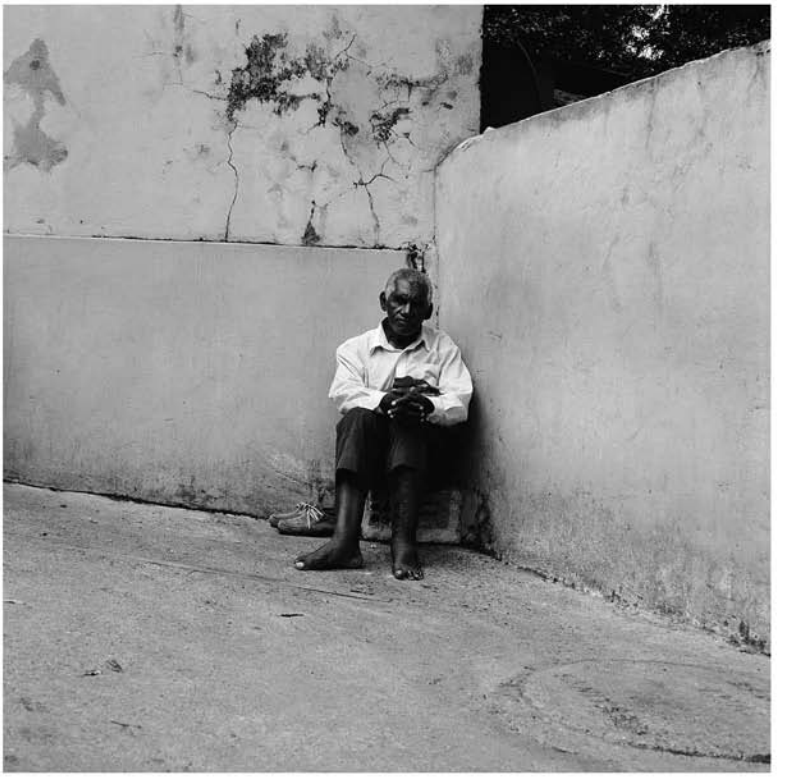
As I came to realize over time, the accounts of many of the so-called mad in Vita were not ever-shifting. Rather, these narratives maintained an impressive steadiness and contextuality (as I would learn by tracking Catarina's history), in spite of caretakers' repeated insistence that they were "nonsense." Instead of seeing these condensed accounts as proof of "a retreat from the world" (Desjarlais 1994:897), I began to think of them as pieces of truth—let me call them life codes—through which the abandoned person attempted to hold onto the real. As I listened, I was challenged to treat the accounts as evidence of the reality from which the abandoned had been barred and their failed attempts to reenter it. In this sense, these pieces ultimately gave language to the exclusion that was now embodied. Moreover, for the abandoned themselves, these accounts were spaces in which destinies were rethought and desires reframed.[35]

Consider the old black man standing barefoot against a wall and how he transformed social dying into speech. As I passed by, again and again he called me *senhor* (master) and, with downcast eyes, begged: "*Senhor*, can you, please, loan me your wife so that I can take her with me to see God, who has descended there in Porto Alegre?" I knew that he had seen me with Adriana, but I did not have a clue as to what he meant.

One day, I asked him about this epiphany.

"God is near the bus station," the old man replied.

How do you know?



Osmar, Vita 2001

"I don't know. People tell me. I also heard it on the radio. I have heard it for some time. Yes, *senhor*, I just want to go there and see God—and if you were to loan me your wife, I could go and see God."

Why do you want to see God?

"It's good to see God, to see and to get to know God."

After a silence, I asked where he had come from.

"I am here now."

How long have you been here?

"My family does not want me to see God, who has descended there at the bus station."

Did your family leave you here?

"My family does not want me to see God. They say that I am not worthy of anyone. That I am worth nothing, that I am a very bad nigger. Truly, that's what they say."

Why did they leave you here?

"I don't know; I didn't wrong anyone."

What did you do before getting to Vita?

"I always worked in a plantation . . . to sell for the market."

How old are you?

"I don't know. I have no birth certificate."

He did recall his name: "Osmar de Moura Miranda." Osmar said that he had always been a single man, that since he was a child he had known "nothing" of his parents. Later, a volunteer told me that Negão—"Big Nigger," that is what they call him in Vita—had actually been brought there by "his boss."

Now Osmar is a useless servant, so to speak; his family is that which never existed; and God (as I interpret it) is the dignity and liberty he never knew as a man of African descent in Brazil. "They brought me here where there is no work to do, and they never let me free. . . . *Senhor*, could you arrange things for me to leave?"

I explained that I could not, but I asked him where he would go if he left.

"To the streets, for I don't have a place to live."

Why is living in the streets better than here?

"In the streets, there is nobody to order me around."

The accounts of Catarina, Osmar, and their neighbors represent agency. As these bits and pieces give language to a lived *ex-humanness*, they also work as the resources and means through which the abandoned articulate their experience. According to the *Oxford English Dictionary*, the adjective "ex" means "former, outdated"; the preposition "ex" is used to mean "out of" in

reference to goods; and the noun "ex" refers to "one who formerly occupied the position or office denoted by the context," such as a former husband or wife. "Ex" also means "to cross out, to delete with an x," and stands for the unknown.

I spent the months of August and December of 2000 working with Catarina in Vita. We talked for hours and hours, and I continued interviewing her neighbors and caretakers. During and between these visits, Catarina finished two more volumes of the dictionary, and I was increasingly amazed by her capacity to give form to her interior life, despite the crumbling of all hope. "I desire to be present"—that is what I heard her saying, time and again.

How to methodologically address her agonistic struggle over belonging? In the simplest of terms, for me it meant first to halt diagnosis, to find time to listen, to let Catarina take her story back and forth, to take her voice as evidence of a relatedness to a now-vanished lifeworld, and, throughout, to respect and to trust.

"Love is the illusion of the abandoned"

Crawling on the cement ground, a man was shouting: "O devil, eat shit! O devil, stick this bread in your ass!"

Most of the people in the infirmary sat quietly against a wall, absorbing the weak warmth of the winter sun on that late morning, August 5, 2000. Some moved around the body of the cursing man and, holding their sole possessions, wandered through the courtyard undisturbed. Inside of me, the man's voice named the place: Inferno. He kept shouting the same thing.

Does he shout all day?

"It's the spirit of the sufferers," replied Catarina.

Do you believe in spirits?

"Yes," she said, "in spirits, in a person passing desire to the other. . . . Like in the church, the priest transmits sexual tension." Catarina added that she had been calm for the past three days, that her tooth was aching less, and that she had been writing "nonstop." Her new volume began: "My dictionary, my name."

On the next page, Catarina wrote: "Clóvis Gama, Catarina Gomes, Catarina Gama."

She again told me that she and Clóvis, the nurse I had not yet met, were "together." After Clóvis's name, she first wrote her maiden name (her husband's family name was Moraes) and then took up Clóvis's last name, Gama—*gamado* means "very attracted to" and "sexually aggressive." Catarina wrote the divorce into her name, I thought. She first identifies with the dead father's name, as if she did not need to belong to another man, but then imagines that she belongs to the nurse who medicates her.

There was now an openness to her name as well. In the dictionary, the

"R" in CATARINA was often replaced by an invented character similar to the letter "K" (which is not part of the Portuguese alphabet)—CATAKINA. She explained, "If I didn't open the character, my head would explode."

These names were followed by an accurate recollection of the Ten Commandments. Then, among a long list of diseases, she had written the statement "rheumatism in the nerves, in the muscles, in the flesh, in the blood" and a reference to "*mal de parto*" (a complication of childbirth) and "amnesia." The next two pages were filled with references to money: "millions of cents, of *reais*, of dollars; Brazilian bank, credit union, savings account." A long list of professions followed, beginning with "doctor and nurse."

The next page, after references to religion, medicine, money, and the body, contained an acknowledgment of sexual enjoyment. She wrote of the discovery of love in sexual tension:

Sexual tension, pleasure, fuck
I discovered that I love you
In every kiss, in every hug
I feel that I want you ever more

In searching for love in the Other's body, she is left with incompleteness and a surplus of desire.

In the next fragment, Catarina wants sperm and confounds the substance with the man:

I desire sperm
Viscous mucus
Now I know you

Love, sex, and the fantasy of both are not distinguishable in Catarina's writing:

Love, fuck, masturbation
Making love with the fingers
Love among two is potency
For then man and woman
Don't feel despised and abandoned

Without social ties, I thought, Catarina was left with sex as if it were love in Vita. Or perhaps she was knowingly writing that one is alone in love, in

sex. One can also read this fragment as suggesting that a sexual relationship writes itself. The spirit of two lone sufferers and sexual tension meet in the discovery of love: an energy or potency, as she put it, to fight abandonment. "We don't know what it means to be alive except for the following fact, that a body is something that enjoys itself" (Lacan 1998:23). In love, in sex, in writing, Catarina approached the reality of being a thing left to die:

To live masturbating and things
Things in life
To alleviate the penis
To be a thing of resurrection

How to be a thing of resurrection? Catarina knows the hatred of which the despised man is made and what he does to the Other as he eases the penis that he is.

Man without God, man without family. The despised man hurts.

Love for Catarina was a vital enjoyment. The dramatic effect of love, she wrote, was a point of suspension in her "nonexistence" in Vita, a path both out of and back into abandonment.

To feel love
Lonely love
To follow desire in lonesomeness
Love is the illusion of the abandoned

Social Psychosis

"Shut up. I am ordering you to shut up." A volunteer wearing a white coat approached the cursing man and threatened to lock him up.

The man on the ground was undeterred: "O devil, eat shit! O devil, stick this bread in your ass!"

As the volunteer turned, he saw me talking to Catarina and walked toward us. It was Clóvis, the nurse. He said he had heard a lot about me, and he apologized for all the noise. "Only medication makes this poor thing shut up. We have to sedate him. But he spits it out. What to do?" Clóvis also apologized for being unshaven. "I really don't have time. Work here begins at 5 A.M. and continues nonstop till 9 P.M."

"Clóvis gives me the vitamins," interjected Catarina.

"Yes, I am the one who medicates . . . when there is medication to be dispensed," added the fifty-four-year-old man. "I give injections, take care of wounds, bathe the grandmas—everything here happens with me." Clóvis said he had been doing this work for almost a year "out of charity. I don't get paid."

He explained that he had learned all these nursing skills while working as a volunteer in Porto Alegre's major psychiatric hospitals. In an association that sounded strange to me and that linked his work to Catarina's, Clóvis alluded to a "pharmaceutical dictionary" he used to read at the São Paulo Hospital, which gave him "the knowledge to now manage Vita's pharmacy." Clóvis was also the man who had thrown away the second volume of the dictionary that Catarina had entrusted to him.

"But let me tell you the truth," stated Clóvis. "I have been an alcoholic for almost my entire life." He began drinking as a teenager. At the age of fifteen,

he left his mother's home in Porto Alegre, becoming a vagabond and a migrant worker. He made brief references to having lived in Uruguay, Paraguay, and Venezuela. For a while, he said, he had a family of his own in Rio de Janeiro, but his only son had died in a car accident.

As we talked, Clóvis disclosed that he had in fact been an inmate in the psychiatric institutions where he had learned about medications and that in the early nineties he had also lived in Vita. "This time, I came back by myself. São Paulo's social worker is my friend; she wanted to rent a place for me, but I decided to come here. My treatment there had ended, and I was afraid that I would go back to drinking and living in the streets. It's tough—alcoholism is a bad disease." This man's history held much more than his words were ready to tell, I thought.

Like many of the other volunteers and inhabitants of Vita's recovery area, Clóvis saw the dying creatures in the infirmary as important material for shaping the citizen he wanted to become: "I can see the inhuman conditions here. So I can tune in, find myself again, and forget drinking. Working with the abandoned is a therapy one does with oneself. I still want to get some more years of work outside here so that I can retire and get my social security benefits. We don't get paid anything here."

As the man on the ground continued to call out to the devil, Clóvis insisted that we move to the pharmacy. He tried to joke with Catarina, saying, "She fights with me. If I don't give her attention, she cries like a cat." Catarina nodded and mentioned something I could not understand.

The infirmary's small pharmacy was indeed well organized now. More medications were on the shelves, mostly donations (many of them expired, as I later verified), and labels were everywhere. On the table, little plastic cups marked with patients' names or nicknames contained the various doses to be consumed throughout the day. Clóvis's services were in high demand. Many men from the recovery area, as well as mothers and children from the village surrounding Vita, knocked at the pharmacy's door, asking him to fill prescriptions or provide medical advice.

"Before I ran the pharmacy, many more people died in here," Clóvis asserted.

How so?

"Wrong medication. I guess that some twenty people died here in one month about three years ago. The police even investigated. It was during the time Vita's administration changed." According to Clóvis, many of his predecessors in the pharmacy "couldn't even read and simply put pills in the cups and distributed them to the dirt poor. Many had heart failure."

Things in Vita changed under the new administration, Clóvis observed. "There are fewer *abandonados* in the infirmary. I shouldn't say this, but I will: Vita now works more like a business. There are still some people from the time of Zé das Drogas, but they are dying out. They accept fewer people into the infirmary now, and the new inmates are mostly elderly women who have pensions. Some of them even get three minimum monthly salaries."

Vita had an organized triage system these days, coordinated by social worker Dalva. "Before," explained Oscar, the infirmary coordinator, "people who came in here had no identity. We didn't know whose children they were or whether they were parents. . . . Many came alone. Now at least they have to have an ID card, and the triage makes it possible for us to contact the patient's family if we have to."

Captain Osvaldo was straightforward about the institution's current modus operandi when I spoke to him later that day: "We are not taking in injured persons anymore. . . . We want to reach a level of zero confinements in the infirmary. We are not a hospital. We are basically a rehabilitation service, so that people can return to society and work."

I asked Oscar to elaborate on the rationale for this shift in policy. Tired of having his pledge to improve the infirmary dismissed, he reluctantly spoke his mind: "Today, the people who administer the institution have their own private interests. This is a political game. They are not interested in the social good of the persons. They are more interested in the visibility the institution has to the outside world. Given all the donations, life in the infirmary should be much better than it is, in terms of food and treatment. I am tired of fighting. . . . They are concerned with the image of Vita outside, while the reality in here is completely different."

As for the future of the infirmary and its inhabitants, he said, "the vision of the administration that the captain passed on to me is that these people will be terminating themselves until we have zero of them. . . . Perhaps in a few years, none of these people you have been talking to will be left." As if it had all been self-generated, I noted, and with no accountability for their suffering. Oscar made an affirmative gesture. "How else? This is the social reason: these people don't produce, they don't vote."

Oscar told me that I was standing in front of "a shattered man," but that he "would move onward with life, with the help of our Lord." He had recently found out that that he was HIV-positive. "AIDS was not the thing I feared most," Oscar said. "I was scared to death that my wife and child would be infected as well. They didn't do anything wrong, like I did, all those years of injecting drugs. But maybe I got infected here in the infirmary as I han-

dled AIDS patients. I finally had the courage to tell my wife, and she was tested. Thank God, she and the child don't have the virus. She told me that she would not leave me, that she would face it with me."

With the help of Vita's social worker, Oscar was learning to navigate the local medical AIDS world. He was being seen by an infectious-disease specialist and had his CD4 levels checked regularly. "The doctor said that I am healthy, that I don't need the cocktail yet, and I will do whatever I can to postpone using it. But thanks to the government, we now have hope." Oscar was referring to the free distribution of antiretrovirals by the Brazilian government.

For the abandoned, however, medical assistance remained minimal. A doctor-philanthropist was still visiting Vita once a week, basically spending one or two hours signing disability reports for the inmates of the recovery area. Depending on the availability of transportation and the good will of volunteers, some of the diseased in the infirmary found immediate treatment at a nearby public health post. Specialized treatment—for cancer or diabetes, for instance—"is a lottery," explained Alencar, a volunteer who, along with hundreds of other people, stood in line from 5 P.M. to 8 A.M. at the university hospital every week, waiting for a ticket to schedule an appointment. "And it generally takes a few more months to see the doctor," he added.

At the local health post, automatic prescription was the rule, and continuity of care was nonexistent. As Clóvis put it: "The psychiatrist prescribes for us. If the medication does not work, I change the dosage and try other combinations." In other words, the man Catarina was attracted to played the doctor, the pharmacist, the nurse, and the caretaker. I wondered who else the man was. "I have a whole bunch of psychotics here," said Clóvis, pointing to all the psychiatric medications he dispensed: "Haloperidol, levomepromazine, chlorpromazine, prometazine, biperiden, diazepam, imipramine." In this utterance, the psychotic was the medication itself.

During my first visits to Vita in 1995, I had heard no talk about "psychotic patients." At the time, the abandoned received no specific diagnoses at all. As Catarina's own account hinted, the widespread availability of psychiatric drugs on the market and through the universal public health service had increasingly framed these creatures as "mental" in family and neighborhood circles as well as in Vita.

As I interpreted the captain's words that day, the rhetoric of mental health also suited the current management priorities of the institution: "As I mentioned before, we try not to bring injured people in, so that we might reach a patient list of zero. But we cannot throw the patients we have back onto

the streets. These dependent beings, the mentally ill, are an obstacle for their families, a financial obstacle, a shame, a social obstacle. But, ultimately, they are not our responsibility. There are specific institutions for that kind of treatment. We don't have adequate personnel for that."

I am concerned with how novel conjunctions of kinship, public institutions, psychiatry, and medication work, if not to make people psychotic, then to give a certain form and value to their experience as psychotic, thus recasting intersubjectivity and mediating abandonment. This is what I mean by social psychosis. I am not saying that mental illness is not real or that psychopharmaceuticals do not carry potential benefits for the afflicted. My point is simply that ethnography can help us to resituate and rethink psychopathology within these various circuits and struggles over belonging, agency, and care. In doing so, it can give us a clearer understanding of the concrete instances in which lives are scripted as otherworldly and out of time and of the work that is needed to give the afflicted and their kin—or, for that matter, health professionals and public officers—another chance and a sense of possibility rather than exclusion. As mentioned earlier, I am particularly interested in assessing the process by which medication becomes a social tool, as this process was recounted by Catarina and the volunteers at Vita: from the doctor's use of medication in treating the working poor at local health posts and in total psychiatric institutions to the role of drugs in the family's recasting of its ties and values, to the individual's chemical and subjective alterations, to Vita's recycling of leftover medication to ensure the removal of the abandoned from reality.

As Oscar described: "I recently hospitalized a grandpa at the Caridade Psychiatric Hospital. He had been here for six years. I don't know where he came from. I stand and look at him: where did he originate? He mentions that he had a family, that he raised cattle. What if all that he says is true? Anyway, he was aggressive and we hospitalized him, but they almost killed him there. If I look back, I don't think he had psychiatric problems. They filled him with medication."

At public psychiatric institutions, added Oscar, the use of medication is indeed undifferentiated and tentative. Idiosyncratic experimentation with new drugs and dosages and oversedation (ostensibly to prevent aggression) becomes the rule, creating a passive patient population that makes it easier for the institutions, with typically weak infrastructures, to continue functioning. In these familial and medical exchanges, pharmaceuticals substitute

for the lack of social links. Moreover, pharmaceuticals make this loss of links irreversible and legitimate the disposal of unwanted persons.

One can argue that in their deployment, psychopharmaceuticals constitute the register of the true and the normal and are integral to the shaping of identity. Biomedically known, Catarina was left without the choice to live her life as she had known it. She was now, in her own words, "on a path without an exit."

As Clóvis left to attend to an injury, Catarina expressed a remarkable understanding of her condition. "I am not a pharmacist," she said. "I cannot say which medication heals an illness, I cannot say the name of the *fârmaco*, but the name of my illness I know. . . . How to say it?"

Say it.

"Mine is an illness of time."

I don't understand, I said.

"Time has no cure."

Sassá, Vita 2001

Lurdes, Iraci, Catarina, and João, Vita 2001